

Workbook – Lesson
Family Systems

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301-3 – Family Systems

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Presented by Tanya Nord, MA, CHt, the Family Systems model provides the Hypnotherapist with an alternative outlook on the client's symptoms as functional in the context of the subconscious forces inherent in romantic and family relationships. This understanding opens the door for HMI graduates to work in conjunction with LMFTs and LCSWs, whose clinical viewpoint originates from this basic tenet.

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Workbook - Family Systems

Learning Objectives

As you complete this learning area, you should be proficient in the following areas:

Knowledge

- The traits of a dysfunctional family.
- The four main roles of children in a dysfunctional family.

Comprehension

- Co-dependency.
- Family stages of development.

Application

- Three approaches to family system.

Learning Approach

The recommended approach to learning is to:

- Refer to the Workbook for this lesson while watching the lecture, taking notes as needed.
- Review your notes, Workbook pages, and class streaming videos.
- Complete the open-book quiz.
- Practice the techniques covered in this class.

Resources

- Your Class Lecture
- Workbook and Notes
- Suggested Reading
- Supplemental Videos

Workbook Table of Contents

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Traits of a Dysfunctional Family

- Dysfunctional families are characterized by the defense mechanism denial.
- There is usually emotional, physical, or sexual abuse.
- There is shame in parents, family, and self.
- Dysfunctional families are created when dysfunctional individuals get married.
- A dysfunctional family keeps family secrets. They put up a front to the outside world and to each other. This characteristic has led to the observation, “We’re as sick as the secrets we keep.”
- Because all systems strive to achieve homeostasis, when changes occur in one part of the system, other changes will follow in another part to maintain the status quo.
- Enmeshed relationships, lack of personal boundaries.
- Feelings are internalized. Free expression of feelings is discouraged.
- Manipulation to get needs met and to control others is common.
- There is a grim atmosphere within the family. Communication and openness are stifled.
- Relationships become very dependent.
- Intimacy problems in relationships are the norm. They can’t trust people or get close to them.
- Chaotic family structure and unclear rules.
- Rigid attitudes
- Reverence for the past and tradition.
- Frequent illness.
- Jealousy and suspicion.
- Triangulation often occurs when there is conflict within the family; it involves bringing a third person into the primary relationship, typically seen when parents communicate through their children instead of directly with each other. This can exacerbate the dysfunctions within the family system.

Traits of a Healthy Family

- In a healthy family, differences are encouraged.
- Self-expression and communication are present.
- Rules are explicit and clearly understood.
- There is respect for the individual with individual freedom.
- Consistent value system.
- Open-mindedness.
- New traditions are encouraged.
- Health
- Independent growth fostered.
- Trust and love are present.
- Self-worth within members of the family.
- Flexible boundaries.
- Pleasant atmosphere.
- Individual personal growth is encouraged.

Roles of Children in Dysfunctional Families

Children develop four basic roles in a dysfunctional family.

1. **Hero:** This is usually the role taken by the eldest child. This person is a high achiever, always doing what is right. Type “A” behavior. Workaholic. They usually marry dependent people whom they can control and manipulate. They have a high need for approval. They feel inadequate. They can never do enough. There is a lot of anger, denial, and fear going on. They suffer from low self-esteem. Very prone to suicide. They fill the self-worth role within the family through achievement.
2. **Scapegoat:** This is the problem child. There is a lot of acting out the underlying family problems. All the dysfunction of the family comes out in them. Hostility and defiance are common, as are trouble with authority and the law. They feel very inadequate. No self-esteem at all. They feel rejected. They give up easily. As an adult, they often become an addict or alcoholic. They have legal problems and are always getting in trouble.
3. **Lost Child:** This is the loner, the daydreamer. Nobody notices this child. They are high emotional sexual. There are issues of abandonment and pain of being unloved and rejected. The lost child is not allowed to have feelings. They are comfortable being by themselves. They have a hard time making friends. They have feelings of loneliness and inadequacy. They have little awareness of their anger. As an adult, they may become promiscuous. They often die early. They lead unhappy lives. They are indecisive.
4. **Mascot:** This is the clown of the family; anything for a laugh. They do things for attention and love. They are fragile, overanxious, and have a high need to be protected. They also have a low self-esteem and feel unimportant. They don’t know why they are here. They usually marry later in life. They withhold a lot of secrets. As adults, they can’t handle stress. They are screaming inside. They are dependent, so they marry the HERO.

In the above four roles, the parent’s dysfunctions are being acted out in these different behaviors. These roles can overlap.

Family Systems

In the family system approach, the family is viewed as a unit. The family is to be taken as one total organism with interaction and interdependence. In order to change the individual, you must look at and deal with the system. In combination with the family system, a personal and social system also affects development.

A child who is the focus of a family system can’t leave the system. He/she becomes enmeshed with the system.

A teenager getting into drugs or other kinds of trouble could be acting out the dysfunctions of the marriage. He/she becomes an addict to keep the family together. His/her unconscious purpose is to try to get the family help.

The identified patient is the individual expressing the symptom of the dysfunctional family. They are expressing a symptom of the system. If the marriage and family are dysfunctional, there is greater chance that the children will also be dysfunctional.

Physical and Sexual Abuse

A child who witnesses violence is a victim of violence. Person's self-worth diminishes when they are beaten or severely physically punished. This can also lead to the condition known as learned helplessness, where the person stops trying because no matter what they do, they can't win. They believe the situation is hopeless, so they give up.

A child will often bond with the offender and then grow up to be a child abuser. In therapy, suppressed rage and anger can surface regarding the non-offending parent (why didn't they protect me?).

Sexually abused children often become compulsive adults. They may express their addictive behavior in many ways, such as through alcohol, drugs, gambling, sex, work, exercise, or eating disorders.

Problems Associated with Adults from Dysfunctional Families

People coming from dysfunctional families tend to find each other. Dysfunctional families produce dysfunctional children who grow up to be dysfunctional adults who produce dysfunctional children.

Emotional divorce is common in dysfunctional families. There are rigid boundaries within the system with the unspoken rule that the couple agree not to disagree. They avoid areas that are potential disagreements and thereby block open communication and the possibility of growing closer. They have a marriage that looks good to the world, but it is really based on "pseudo-intimacy" (the illusion of intimacy and health). It is really filled with pain, disappointment, frustration, bitterness, and loneliness.

There is often a strong need to be perfect, which can lead to compulsive behaviors. Procrastination can come about because of the fear of failure or fear of success. Hostility and rage, or passive-aggressive behavior, is common. There is often a lot of thinking of self or others as objects. Possessiveness, jealousy, and other controlling behaviors are typical.

Abused children grow up with problems dealing with trust. They can't trust people because their trust was broken by a family member. As an adult, they can't trust their feelings or thoughts. Either the person will avoid touch, or they will be promiscuous. They tend to have poor physical boundaries, free floating anxieties, and shame-based needy marriages.

Co-Dependency

Taking care of another person's needs over your own and/or a preoccupation with, or an overdependence on another person's needs or problem.

An example would be the spouse of an alcoholic working hard to cover up for and protecting the alcoholic (i.e., the wife calling to say that her husband is unable to work due to illness). It is behavior characterized by denial. Co-dependents are usually fixers or rescuers. They look for people to fix (alcoholics, addicts, etc.). The co-dependent is dependent on this as much as the alcoholic is on alcohol.

Co-dependents have trouble being spontaneous. They have problems being intimate. They often don't know what normal behavior is because of poor modeling. They have an exaggerated need for approval from others. They tend to look for other people to care for. They need to control themselves and others.

Co-Dependents in Therapy

The therapist should act like a feedback loop on how they are denying or avoiding their problem. Have them tell the story again, but this time without the smile. Look for their strategy for survival. Figure out which role they take on. Don’t push them too hard; they will run if you push their façade of competence. Go slow. Don’t go for cause too soon. Deal with symptoms until they bring up the cause.

They will try to be the perfect client. They want your approval. They are trying to please you. They missed their childhood. They are adults on the outside and children on the inside.

Give them love, acceptance, and nurturing. Build their trust, raise their self-esteem, and then you can deal with other issues. Give them relief from symptoms so they can go on to other issues. Get them to give up their family secret.

Other High Stress Family Systems

1. Families that display compulsive behavior such as:
    - Compulsive eating or dieting
    - Workaholism
    - Gambling
    - Exercising
    - Spending
    - Cleaning
    - _____
2. Families with constant arguing and tension.
3. Families in which parents refuse to speak to each other for extended periods of time.
4. Families with parents who have conflicting attitudes and values or display contradictory behaviors that compete with their children’s allegiance.
5. Families with parents who are competitive with each other or with their children.
6. Families with a parent who cannot relate to others in the family, and thus, actively avoids them while blaming them for this avoidance.

Three Approaches to Family Systems Therapy

Structural: looking at general family structure, coalitions, dyads, and triads.

Communication: how we communicate; two levels of communication:

Surface level: verbal. Metacommunication: non-verbal, body language, tonality…

Hierarchy: complimentary relationship: one up one down Symmetrical relationship: equality

The further apart they are, the more dysfunctional is relation.

Family Life Cycle, States of Development

1. Between families: young adult in the family. Unattached young adult searching foe own identity.
2. Newly married couple; learn how they relate to each other, create rules, and structures.
3. Family has a child: family with young children (from a structural point, dyads and triads are forming.
4. Children – adolescents, unique and different stages.
5. Launching children, helping kids going on their own.
6. Family in later stage.

Working with the Family Unit

IP – identified patient; person with presenting problem, which is a metaphor for family unit dysfunction

1st order change: creating change in IP presenting problem, working with symptoms.

2nd order change: creating change in the family unit.

Cognitive Approach to Create Changes

1. Changing communication style
2. Prescribing symptoms
3. Time out
4. Scheduled arguments
5. Pretend
6. Differentiating feelings from the past and present
7. NO criticizing, defending, demeaning

Anger should be expressed if it is not violating other people’s rights.

Continuing Education Opportunities

Suggested Reading

1. Amazon.com: Adult Children of Alcoholics: Expanded Edition
   by Janet Geringer Woititz
2. Amazon.com: Bradshaw On: The Family: A New Way of Creating Solid Self-Esteem
   by John Bradshaw
3. Amazon.com: Leaving Home: The Therapy Of Disturbed Young People
   by Jay Haley

Supplemental Videos

1. ELE: Counseling and Interviewing – Online Course
2. PPV: Hypnosis for the Caregiver – Online Certification Course
3. American Hypnosis Association – Media Library

Recommended AHA Events Live on Zoom

- EVE: Hypnosis and PTSD – Certification Course

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